Auth- MD6709014

[redacted]

[redacted]

DISCHARGE AND FOLLOW-UP CARD

चिकित्सालय से छुट्टी प्राप्त करने तथा पुनः परीक्षण हेतु बुलावे का पत्र

DEPARTMENT

Orthopaedics

NAME [redacted]

AGE/SEX 74 y/M.

Admission No./OPD No. 92105260026 1288

C.R. No./IPD No. 4868.

DATE & TIME OF ADMISSION

(i) (ii) 2/03/26 @ 12.00 PM (iii)

DATE & TIME OF DISCHARGE

(i) (ii) 9/03/26. @ 4:00 PM. (iii)

UNIT INCHARGE [redacted]

SERVICE UNIT

PROGRESS SHEET

C.R. No. .....

Name ..... Age/Sex .....

Consent for Surgery

हमें हमारे आपका में समस्या दिया गया है कि हमारे मरीज को Osteoarthritis है Left knee को जिसके लिए आपरेशन की आवश्यकता है। हमें इसके कारण व लक्षण समझा दिए गए हैं और इस अपनी मर्जी से या दबाव करवाना चाहते हैं।

Risks involved

1. Pain, swelling, stiffness
2. Infection / bleeding
3. Injury to surrounding Nerves or bloodvessel
4. Delayed union / malunion
5. Implant failure
- Limb length discrepancy
- Anaesthesia related complications

[redacted]

Patient

12

[redacted]

ESS SHEET

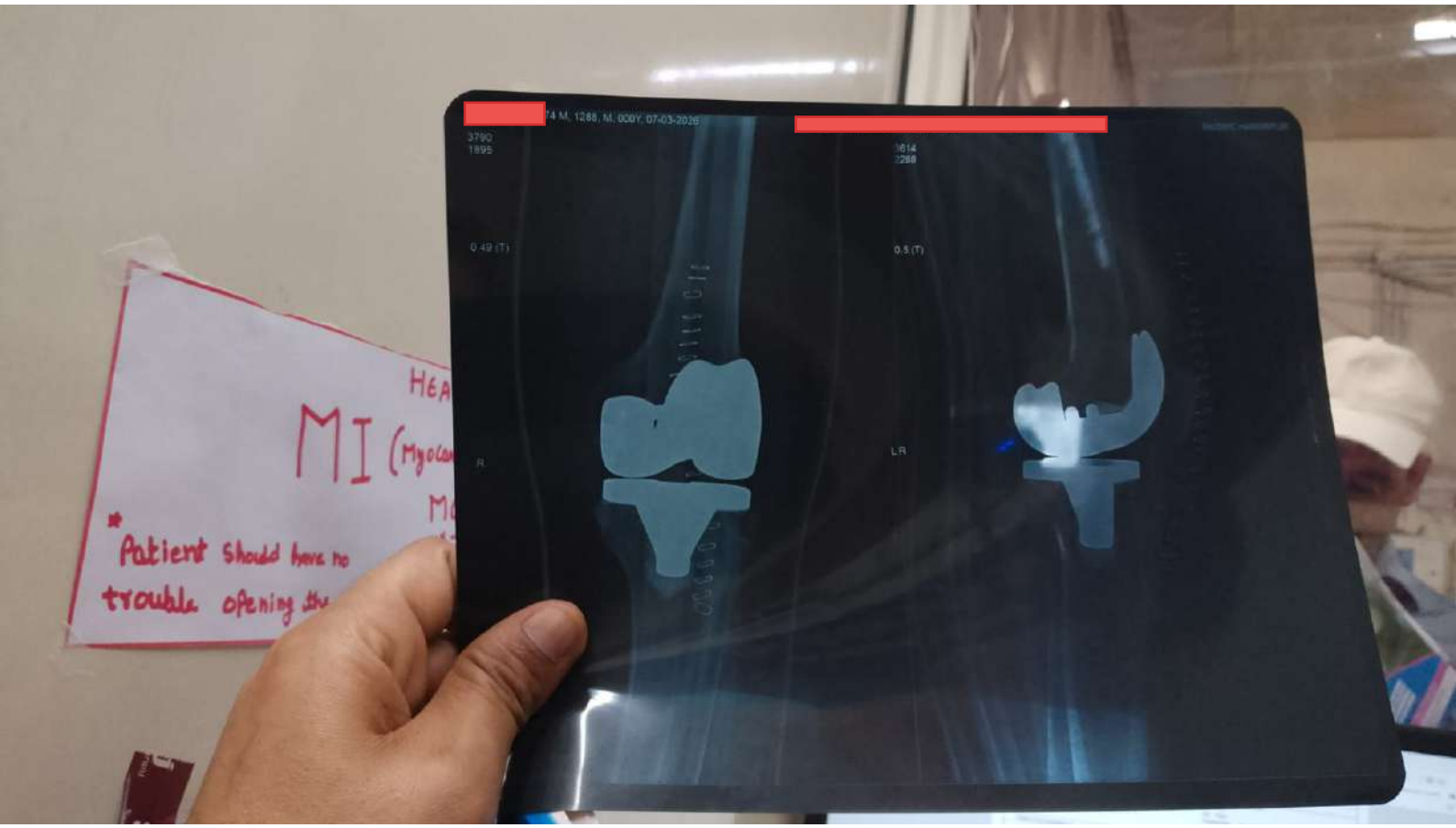
C.R. No. .....

Age/Sex .....

Pre op vitals

→ NPO after 12AM
→ Follow PAC
→ Fresh warm informed consent
→ Inj Ceftriaxone 1 gm IV ATD
— Blood arranged
→ Blood Bu, ECG, viral markers
→ Inj @ 75 m/hr
→ Send pt to OT on call
— Pad Rupanaku

Pre op vitals.
BP → 128/72 mmHg
PR → 62 bpm
SpO2 → 98% @ RA


MI
Patient should have no
trouble opening the
0.49 (T)
0.5 (T)
R
LR

ESS SHEET

C.R. No. .....

Age/Sex.....

LOT A2507300 | REF MK-UFPSLE00

DESTIKNEE™ | Knee System

POSTERIOR STABILIZED
FEMORAL COMPONENT
SIZE E LEFT

UDI
(01)08906067240040
(11)250824
(17)300823
(21)044
(10)A2507300

IN 2025-08-24 | 2030-08-23

Meril Orthopedics | MD | CE 2460 | STERILE EO

Meril Healthcare Pvt. Ltd.
M.L.NO. MFG/MD/2018/000063

Rev.02
MH/AW/KS/030

LOT C2508114 | REF MK-MLPSXC309

DESTIKNEE™ | Knee System

POSTERIOR STABILIZED
TIBIAL ARTICULAR SURFACE
SIZE C3-4 09MM HEIGHT

UDI
(01)08906067240880
(11)250905
(17)300904
(21)128
(10)C2508114

IN 2025-09-05 | 2030-09-04

Meril Orthopedics | MD | CE 2460 | STERILE EO

Meril Healthcare Pvt. Ltd.
M.L.NO. MFG/MD/2018/000063

Rev.02
MH/AW/KS/030

LOT B2509130 | REF MK-MTUUX400

DESTIKNEE™ | Knee System

TIBIAL BASE PLATE
SIZE 4

UDI
(01)08906067241832
(11)250927
(17)300926
(21)019
(10)B2509130

IN 2025-09-27 | 2030-09-26

Meril Orthopedics | MD | CE 2460 | STERILE EO

Meril Healthcare Pvt. Ltd.
M.L.NO.MFG/MD/2018/000063

Rev.02
MH/AW/KS/030

Attach to patient's record
66057851 PALACOS® R
LOT 76061500
2030-05-31
(01)04260102133882(17)300531(10)76061500
Heraeus Medical GmbH, 61273 Wehrheim, Germany

Med. Form No. 7 (B)

TEMPERATURE-PULSE CHART

Name [redacted]

Date 2/3/26

Age

Sex

Diagnosis

Result

Days of Month	2/3	3/3	4/3	5/3	06/03	07/03	08/03	09/03
Days of Disease	1	2	3	4	5	6	7	8
Pulse / Temperature	ME ME	ME ME	ME ME	ME ME	ME ME	ME ME	ME ME	ME ME
160 / 107°								
150 / 106°								
140 / 105°								
130 / 104°								
120 / 103°								
110 / 102°								
100 / 101°								
90 / 100°								
99°								
80 / 99.4/98°								
70 / 98°								
60 / 97°								
50 / 96°								
Pulse								
Respiration								
Bowel								
Urine								

... 2024-21-03-2024—60 पेज्ज।

PROGRESS SHEET

C.R. No.

Name

Age/Sex

Post-op order :- 1) N/O till further order
2) IVF @ 100 ml/hr till N/O
3) O_2 inhalation if SpO_2 < 90%
4) I/O charting & vital monitoring
5) Care of epidural catheter
6) Avoid hot fomentation to anesthetized parts
7) Avoid head lift & unassisted ambulation
8) Inform SOS

Adv - Pulm. med. opinion for peripheral gangrene [redacted]

PROGRESS SHEET

C.R. No.

Age/Sex

OT Notes

3/3/26

CIT :- Osteoarthritis Bilateral Knee Lt > Rt 2° Rheumatoid arthritis

Sx :- Total Knee Replacement (LF) Side

Tibial tray
femoral component E

Dr. Sanjay Thomas
Dr. Naveen
Dr. Pankaj Sreenivas

JCSE
Dr. Shuaibi

POST OP ORDER

NPO x 6 hours
IVF NS - NS maintainance
Inj Ceftriaxone + Sulbactam 1.5 gm i/v
Inj Amikacin 500mg i/v
Inj PCM 1 gm i/v
Inj pantop 40mg i/v
Inj Diclofenac 75mg im SOS

o pulmonary consultation

o O_2 inhalation 2-3 L/min

Hydralg

[redacted]

PROGRESS SHEET

C.R. No. .....

Name ..... Age/Sex .....

Consent for Surgery

हमें हमारे आपका में समस्या दिया गया है कि हमारे शरीर को Osteoarthritis है Left knee को जिसके लिए आपरेशन की आवश्यकता है। हमें इसके कारण व नुकसान समझा दिया गया है और हम अपनी मर्जी से या दबाव करवाना नहीं है।

Risks involved

1. Pain, swelling, stiffness
2. Infection / bleeding
3. Injury to surrounding Nerves or bloodvessel
4. Delayed unions / nonunion
5. Implant failure
- Limb length discrepancy
- Anaesthesia related complications

[redacted]

[redacted]

ESS SHEET

C.R. No. .....

Age/Sex .....

Pre op vitals

→ NPO after 12AM
→ Follow PAC
→ Freely written informed consent
→ Inj Ceftriaxone 1 gm IV ATD
— Blood arranged
→ Blood Bun, ECG, viral markers
→ Inj @ 75 m/hr
→ Send pt to OT on call

— Pad Rupanaku

Pre op vitals.
BP → 128/72 mmHg
PR → 62 bpm
SpO2 → 98% @ RA

[redacted]

Name: [redacted] Age/Sex: 74/F. C.R.N.

Chief Complaints with duration :

1. Pain/swelling Lt Knee
2.
3.
4.
5.

History of Present illness :

Patient was apparently well 20 days back when he started having swelling (L) knee a/w c̄ swelling

DRY

ON

Newly diagnosed HTN. x 2 weeks. on Tab Telmisartan 40mg

No h/o DM, ATT intake

HISTORY

No significant family.

NAL & SOCIAL HISTORY

Non veg. Alcoholic (+) Smoking (-).

TRUAL HISTORY

ETRICAL HISTORY

INATAL/DEVELOPMENTAL/IMMUNISATION HISTORY

OCUPATION/TYPE OF WORK

Retired from PWD.

DDICATION IF ANY

[redacted]

(State Govt. Undertaking)

Regd. Office :- N & T (T) BLOCK S.B.A Complex Shillong-9

CST No: HIM 2284 19.02.81

GST No: SIM-TTT-4120 19-02-81 **CRTNO: 02AABCH4054K1ZV **

**FSSAI NO. [redacted] DL NO. HP HAM 23741 [illegible]

Medicine Shop : [redacted]

CREDIT MEMO

** ORIGINAL BILL **

CASH MEMO No/Dt: 03 [illegible] 20

PATIENT'S NAME: [redacted]

Prescribed By: [redacted]

CR.No :

Prescription No :

-- H.S.N.	Particulars	Company	Batch No.	Exp Dt	Quantity	Rate	CGST%/SGST%	Amount	Disc%	Net Amt
1	CAUTRY PENCIL	REGAL	R 2524401938	08/30	1 PKT.	1226.30	2.5/2.5	1226.30	10.25	1100.68
2	VICRYL 1 NO	G.B.C	V256446A	08/30	3 PKT	949.00	2.5/2.5	2847.00	10.25	2551.18
3	SYRINGE 50ML	ALBERT	548501RITN	07/30	2 NOS.	36.09	2.5/2.5	72.18	30	50.53
4	IOBAN 2 6650	3M MEDI	346K31	09/27	1 PCS	1759.13	2.5/2.5	1759.13	15	1495.16
5	GLOVES 6.5,7,7.5 POWDER	KAMADA	F1594	05/29	4 PAIR	135.94	2.5/2.5	543.76	40	326.26
6	ETHILON 1 0 NO.	LOTUS	V256009	09/30	2 PKT	427.00	2.5/2.5	854.00	10.25	766.44

(-) ITEM DISCOUNT : 100.18

Rs. :Six Thousand, Two Hundred Eighty-Nine Only

GRAND TOTAL Rs. 6289.18

Please Show The Medicines To Your Doctor Before Use. In Case Of Any Inadvertent Error In The Price Charged Or In Calculation Kindly Bring This Memo For Refund/ Correction.

Please Bring ORIGINAL BILL at the time of return. NO RETURN Bill be entertained without the ORIGINAL BILL. Mention your NAME/ PHONE/ ADDRESS on ORIGINAL Bill at the time of RETURN RETURN against this BILL would be entertained till 45 Days from the BILL DATE

BILL Unique Id 144B-7DF9-A33F-42D2-B691-9DA5-22DA-492F

For N.B.S.T.C.L.

C.R. No.

Name

Respected Sir,

[illegible]

3/3/26. Call to DoD Medicine

Respected Sir

BP = 104/55 mmHg
PR = 61/min - regular

Here is a Pt on MOW of Sx (P)

Pt was c/k/c smoker & chft smoker

his surgery patient peripheries were cold & clammy

Pt has sinus bradycardia on ECG

Please evaluate Patient & give your opinion.

CONTINUATION SHEET

Serial No.

Patient's Name

Date	Daily Progress	Treatment
	E.C.G. - Sinus Bradycardia	
Medication notes		CMAT - (BPL vbr(1)) no cesper/Bonalfi
PR = 56 bpm BP = 104/58 mmHg SpO2 - 98%		Rx 1) Nor e levophin 4 hrly get serial ECG after 4 hrs
PLAN - RR & e - PPI		
4/3/26		POD - 1 clinically stable No fever complaints OST

[redacted]

[redacted]

[redacted]

Name :	[redacted]	Lab No :	GDC1256/3
Age/Gender :	74 Years/Male	Sampled Date :	21-02-2026
Ref. By :	[redacted]	Reported Date :	21-02-2026
Reg No :	1256	Specimen :	--

CRP (C-Reactive Protein)-Quantitative

CRP (C-Reactive Protein)-Quantitative	13.0 H	mg/L	< 6

Method: Turbidimetric Immunoassay

INTERPRETATION

The level of CRP in the blood is normally low.

Increased CRP level:

- A high or increasing amount of CRP in the blood suggests the presence of inflammation but will not identify its location or the cause.
- Suspected bacterial infection—a high CRP level can provide indication that patient has an infection.
- Chronic inflammatory disease—high levels of CRP suggest a flare-up if you have a chronic inflammatory disease or that treatment has not been effective.

If the CRP level is initially elevated and drops, it means that the inflammation or infection is subsiding and/or responding to treatment.

RHEUMATOID FACTOR QUANTITATIVE

RHEUMATOID FACTOR, Quantitative	129.5	0-20

Method: Turbidimetric Immunoassay

Interpretation

Approximately 85% of patients with Rheumatoid arthritis have detectable RA. It may also be seen in other medical conditions like Sjogren's syndrome and SLE.

*Increased in RA, Chronic hepatitis, chronic viral infections, cirrhosis, infectious mononucleosis, leishmaniasis.Syphilis etc.

*High levels are also seen in inflammatory rheumatic diseases and in various non rheumatic diseases.

End of Report

[redacted]

Legal Purpose